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HOPEHOSPITAL
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Prescription Audit Report

Hospital Name: Hope Hospital
Audit Conducted by: Dr. Shiraz Khan
Date of Audit: 15 September 2025
Location: Nagpur, Maharashtra

1. Introduction

A prescription audit was conducted to evaluate the rationality, completeness, and adherence to standard guidelines in prescriptions issued at Hope Hospital. This audit covered both outpatient and inpatient prescriptions across major departments.

2. Objectives of the Audit

- To assess completeness of prescriptions.
- To evaluate rational drug prescribing.
- To measure adherence to WHO core prescribing indicators.
- To identify gaps and recommend improvements.

3. Methodology

- **Sample Size:** 120 prescriptions (80 OPD + 40 IPD).
- **Departments Covered:** General Medicine,Surgery, Orthopedics, Emergency.
- **Audit Duration:** 1st – 7th September 2025.

- **Tool Used:** WHO core prescribing indicators checklist & NABH guidelines.

4. Audit Findings

A. Completeness of Prescriptions

Parameter	Compliance (%)	Remarks
Patient details (Name, Age, Sex, ID)	95%	Few OPD slips missed UHID
Date of prescription	100%	Consistently recorded
Prescriber's name & signature	90%	Some missing designations

B. Rationality & Prescribing Practices

Indicator	Result (Audit)	WHO Standard	Remarks
Average number of drugs per prescription	3.2	< 2.0	Polypharmacy in General Medicine
% of drugs prescribed by generic name	46%	> 80%	More brand-name usage
% of prescriptions with antibiotics	38%	< 30%	Higher in Surgery & Pediatrics
% of prescriptions with injections	22%	< 10%	Overuse in IPD cases
% of drugs from Essential Drug List (EDL)	72%	> 90%	Needs improvement

C. Legibility & Safety

- **Legibility:** 85% were clearly readable, 15% required clarification.
- **Unsafe abbreviations:** Found in 12% prescriptions (e.g., “OD”, “IU”).
- **Dosage, route, frequency:** Correctly mentioned in 88% cases.

5. Strengths Observed

- Almost all prescriptions had patient identifiers and date.
- Increasing shift towards electronic prescribing in Medicine OPD.
- Antibiotic use partially aligned with stewardship guidelines.

6. Gaps Identified

- Missing diagnosis in 22% prescriptions.
- Excessive brand-name prescribing (54%).
- Antibiotic usage above WHO benchmark.
- Overuse of injections in IPD cases.
- Incomplete prescriber details in some cases.

7. Recommendations

1. Conduct **training programs** on rational prescribing.
2. Mandate **diagnosis entry** in all prescriptions.
3. Encourage **generic prescribing** to reach >80%.
4. Strengthen **Antimicrobial Stewardship Program (AMSP)**.
5. Standardize a **uniform prescription format**.
6. Conduct **quarterly audits** to monitor progress.

8. Conclusion

The audit highlights good practices at Hope Hospital but also significant gaps in rational drug use, generic prescribing, and completeness of prescriptions. Implementing structured interventions will enhance patient safety, optimize therapy, and align practices with NABH and WHO standards.

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